



Critical Care Billing for Emergency Physicians

The Basics

Unlike typical EM coding, which is based entirely on MDM, critical care is billed based on the **time** you spend providing critical care services.

What Counts as Critical Care Time

Critical care time includes the time you spend providing care to a critically ill patients, but it also includes time spent on activities directly related to that patient when you're not at the bedside.

Time you **CAN** count:

- Bedside evaluation and treatment
- Review and interpretation of labs, imaging, and monitoring data
- Documentation
- Discussions with consultants, nursing staff, or family
- Time spent on the phone arranging transfers or discussing the patient

Time you **CANNOT** count:

- Time spent on separately billable procedures (intubation, central line, chest tube, etc.) — this time is excluded from your critical care clock

Who Meets Critical Care Criteria

The patient must have an acute impairment of one or more vital organ systems that, without intervention, would result in a high probability of life or organ-system threatening deterioration.

Clinical examples that typically qualify:

- Respiratory failure requiring ventilatory support or imminent intubation
- Hemodynamic instability requiring vasopressors or aggressive resuscitation
- Arrhythmia necessitating IV push medications or drips to stabilize
- Septic shock
- Active MI with hemodynamic compromise
- Stroke
- Major trauma with shock
- Severe metabolic derangement (e.g., DKA with altered status, severe hyponatremia)
- Overdose with cardiovascular or respiratory compromise, or needing IV intervention (Narcan)

How to Document It

Your documentation needs to do three things:

- State critical condition and organ system threatened (this is likely in your MDM note)
- List your critical intervention
- State time spent on critical care services excluding other billable procedures.